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Name: هبة محمد احمد حسن National ID: 28803010102627	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

هبة

محمد احمد حسن

١٤ مساكن الزاوية الحمراء ب-٣ م-٣
الزاوية الحمراء - القاهرة

١٩٨٨/٠٣

LP0891019

٢٨٨٠٣٠١٠١٠٢٦٢٧

DMS

٤٠٦١٤١١٣



٢٠٢٦/٠١ ٢٨٨٠٣٠١٠١٠٢٦٢٧

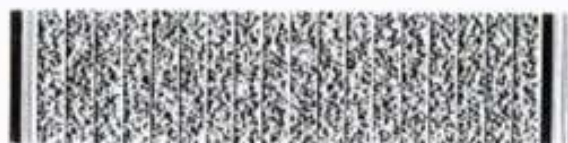
حاصلة على بكتوريوس

نظم معلومات ادارية

التي



البطاقة سارية حتى ٢٠٢٣/٠١/٢٣



Patient Name	: الأستاذة / هبة محمد احمد	Request No.	: 13008838
Patient No.	: 28803010102627	Request Date	: 09/07/2023
Path No.	: 2145/23	Verification Date	: 16/07/2023
Sex	: Female	Financial Category	: أوزام دار السلام - مبادره
Age	: 35 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 09/07/2023, 11:00 am.

Gross Examination:

- Five grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 20% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 20% of tumor cells.

Verified by :

Dr.Marwa mahmoud

Printing Date : 18/07/2023 09:24:13

Page 1 of 2

Printed By : marwa mohamed yousse



Patient Name	: الأستاذة / هبة محمد احمد	Request No.	: 13008838
Patient No.	: 28803010102627	Request Date	: 09/07/2023
Path No.	: 2145/23	Verification Date	: 16/07/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 35 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

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- Ki67: Positive in 20% of tumor cells.

Verified by :

Dr.Marwa mahmoud

Printing Date : 18/07/2023 09:23:52

Page 1 of 2

Printed By : marwa mohamed yousse



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A- Patient ID:

1- Name: مريم محمد أحمد حسن

Sex:

3- Medical number:

Female

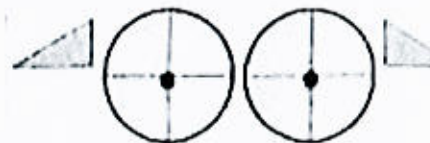
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Old Med:

2685/2023

2- ID number: 26803010102627

Patient Age: 35 Y



Family history

+VE F.H

Phone

01006746439

BMI

25

REASON OF MDT

NEW

marital status

DIVORCED

NBS Score

1

GROUP

4

Date of MDT

2023-08-02

Ce morbidity

Free

Menopausal status

Clinical data

Breast Imaging:

son mammo was done with a diff name

MRI LT UOQ & LOQ LABC + path. LN

Mets Workup:

CT: FREE

BX:

IDC

GIT

ER:

8/8

PR:

8/8

Her 2 Neu:

0

SISH:

KI67:

30%

T
3

N
1

M
0(ct)

Sonomammography

son mammo Was done with a diff name

MRI LT UOQ & LOQ LABC + path. LN

PATIENT HISTORY

CLINICALLY LABC FOR NACTH THEN MRM

1-BONE SCAN

2-ECHO

3-LABS

4-BRACA testing

MDT OUTCOME & PLAN

start NACT (AC x4---->taxanes x4) +zoladex then consider it mastectomy & ALND +/-RECONSTRUCTION

SIGNATURE

MEDICAL ONCOLOGY

د. محمد أحمد حسن

RADIOLOGY

د. محمد أحمد حسن

SURGERY

د. محمد أحمد حسن

PATHOLOGY

د. محمد أحمد حسن

Psychiatric

Nutrition

د. محمد أحمد حسن



Patient Name: Heba Mohamed Ahmed Hassan	Date : 26/10/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 36 Year(s)
Medical ID: 2685/2023	Pat. ID: 39784

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- ACR:D
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Youmna tarek

26/10/2024 1:36 PM

Nuclear Medicine Unit

خطابات أورام

Patient Name: Heba Mohamed Ahmed

Date: 11/5/2025

Referring physician: Dr. T. El-M **99mTc-MDP BONE SCAN**

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No active osseous metastatic lesions could be seen all over the surveyed body .
- Spondylo degenerative changes are seen at the spine ,
- Both kidneys are normally seen.

Conclusion:

No active osseous metastatic lesion could be seen all over the surveyed body ,

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

DR/ Mohamed Emam



NAME: HIBA MOHAMED AHMED
DATE: 16/8/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

- Liver: -**
- Average in size (15cm at Rt MCL) homogeneous parenchymal echo pattern with smooth surface.
 - No hepatic focal lesions or intra hepatic biliary radicle dilatation.
 - Portal vein is patent and not dilated.
- Gall Bladder: -**
- Average distention and capacity with no stones or mud.
 - The wall is not thickened.
 - CBD is not dilated.
- Spleen: -**
- Average size, homogenous.
- Kidneys: -**
- Both are average in size, shape and echogenicity
 - No stones or dilated pelvicalyceal system
 - No gross lesion
- Para-aortic and pancreatic area: show no gross abnormalities.**
- No ascites**
- Urinary Bladder: -**
- Average capacity with no stones or diverticular out pouching
- Uterus and adnexal: no abnormalities could be detected**

Opinion:
Unremarkable study

Best regards
Dr. NERMEEN RAMADAN FEBR, MD

Name: Heba Mohamed Ahmed Hassan
Referred by: Prof.

ID: 992786
Date: 06/07/2023

High Field Open (1.5T) Dynamic MR-Mammography of

Both Breasts

Technique :

- Axial T2 (plate 1).
- Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the right breast (plate 3).
- Sagittal postcontrast 3D TFE of the left breast (plate 4).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5).
- Time intensity curves (plate 6).

Findings:

- Bilateral glandular condensation and evidences of bilateral homogenous glandular enhancement.
- Scattered tiny sized breast cysts in both breasts .
- Left breast upper outer and lower outer quadrant large infiltrative masses surrounding by enhancing satellite lesions extending between the 12:00 to 5:00 locations , the masses measure 50x40 mm , 6x24 mm and 38x33 mm in diameters . the lesions are low signal in T2 and STAIR with intense mass like enhancement . the kinetic data obtained indicates a malignant pattern , rapid rise followed by wash out and plateau , type 2b& 3 curves .the LOQ lesion is seen surrounded by architecture distortion and associated with skin tapering and retraction.
- Pathologically enlarged right axillary lymph nodes the largest measures 22x17 mm in diameters .
- No right breast masses and no architecture distortion.
- Normal skin thickness and contour of the right breast.
- There is multiple likely non-specific right axillary lymphadenopathy.

OPINION :

Left breast UOQ & LOQ locally advanced multifocal carcinoma, categorized as BIRADS 5 based on the morphological and the kinetic data.
Right breast fibroadenosis and tiny cysts , BIRADS 2.

Prof. Naglaa Abdel Kazez, MD



اسم المريض: د. محمد علي
رقم الملف: 123456789
تاريخ الفحص: 18/07/2023
وقت الفحص: 10:28:11 PM
اسم الطبيب: د. محمد علي
اسم المستشفى: مستشفى الملك سعود
اسم العيادة: عيادة الفحص

- The liver is of average size showing segment (IVb) patchy are of non-enhancement (likely pseudo-lesion).
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

II) ABDOMEN & PELVIS:

- Left breast UOQ irregular enhancing mass lesion measuring about 5x2.5 cm associated with few prominent left axillary (level I) lymph nodes; the largest measures about 7 mm in maximum short axis diameter (for sono-mammographic correlation).
- Prominent left internal mammary lymph node measuring about 4 mm in short axis diameter.
- Bilateral upper and right lower lobar apical fine reticulo-nodular infiltrations associated with bilateral lower lobar tiny calcified pulmonary nodules (likely sequel of old granulomatous infection).
- Clear rest of both lung fields. No pulmonary masses, areas of consolidation or cavitary lesions.
- No pleural or pericardial effusion.
- No gross cardiomegaly.
- No pathologically enlarged hilar or mediastinal lymph nodes.

I) CHEST:

Findings:

Clinical data: Newly diagnosed breast cancer for metastatic work up.

ADMINISTRATION:

MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS AFTER I.V. CONTRAST

CT

Visit Number: 71023514169
Patient Name: د. محمد علي
Age / Sex: 35 year / Female
Patient ID: 2131748481

Client Name:
Referred by Prof:
Exam Date: 18 Jul 2023
Printed: 7/18/2023 10:28:11 PM



CT Scan
[DOCTYPE:CT_SCAN]



Dr. Abdulrahman Ibrahim

Dr. Sally Fouad, MD

OPINION:

- No current CT evidence of pulmonary or hepatic metastatic deposits.

- Normal size & parenchymal texture of the spleen with no focal lesions seen. A splenule is seen related to the lower pole of the spleen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus with posterior wall well defined hypodense lesion measuring about 1.4 x 0.9 cm (likely fibroid, for US correlation).
- Right adnexal multiple foci of coarse calcifications.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

CT

Visit Number: 71023514169
 Patient Name:
 Age / Sex: 35 year / Female
 Patient ID: 2131748481

Client Name:
 Referred by Prof:
 Exam Date: 18 Jul 2023
 Printed: 7/18/2023 10:28:11 PM



Referring physician: Dr. MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi $^{99m}\text{Tc-MDP}$:

Report:

- No active osseous metastatic lesions could be seen all over the surveyed body.
- Spondylo degenerative changes are seen at the spine ,
- Both kidneys are normally seen.

Conclusion:

No active osseous metastatic lesion could be seen all over the surveyed body ,

Nuclear Medicine Unit

Dr. Sherin Maky, MD

Dr. Mohamed Zaman



Echo
(DOCT/RECHOC)

Echo report

Name: Dr. Huda A. B.

Date: 28/12/2023

MR : 2685

EF	69%	55-75%	PWT	0.75	2-3.7cm
LVEDD	4.55	3.5-5.8cm	AO.R	1.89	2-3.7cm
LVEDSD	2.76	2-4cm	LA	2.60	2-4cm
FS	39%	25-45%	RV	~	0.7-2.3cm(supine)
SWT	0.75	0.6-1.1cm	AO.CUSP	~	1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function Not Dilated, TAPSE 23.
Intracardiac masses and effusion No effusion in Jaws

D/ Msc cardiology



٢٠١٨/١١ ٢٨٨٠٣٠١٠١٠٢٦٢٧

حاصلة على بكالوريوس

نظم معلومات اداريه

أنثى مملعة متزوجة

سامح جمال رمضان على

البطاقة سارية حتى ٢٠٢٥/١١/٢٦



ERM036725907

فتح صلف نزل بغير
١٩ امان



جمهورية مصر العربية
بطاقة تحقيق الشخصية

هبة

محمد احمد حسن

١٦ مدائن الزاوية الحمراء ٣
الزاوية الحمراء القاهرة

٢٨٨٠٣٠١٠١٠٢٦٢٧

HU3728814



ب.هـ

الرقم الطبي 3153026
لخدمة صحة المرأةب.هـ
م.هـ

MDT FORM

/

/20

G

01002553678

Name	أحمد بن محمد	MRN	6710
Age	35 y	ID	5-57
Menopausal status	premen	Phone	+966 11
NRS SCORE		Marital status	
BMI		Family history	Relative
CO morbidity		REASON OF MDT	BRCA2 mutant

Clinical Data

01065592014

BX: IDC

ER: +v	PR: +v	HER 2: -ve	Ki67:
8/8	8/8		

R

Post-NACT

L

Immunohistochemistry

multifocal

premenopausal left breast

B1



PR

Mets Workup:

e-T3, N, Ho

CRBS - f

PR Rec NACT

4 AC, 1st cycle
Case 23/12/2022

Recommendations:

for MDT

all cases

10/1/22

MDT Outcome & Plan

For 1st MDT (delayed reconstruction due to large breast size if silicone is not available)

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

P.O. /



Patient Name: Heba Mohamed Ahmed Hassan	Date : 03/08/2024
Exam : MRI CERVICAL SPINE	Age : 36 Year(s)
Medical ID: 2685/2023	Pat. ID: 39784

رئيس الرقابة الطبية

I-MRI EXAMINATION OF THE CERVICAL SPINE

Technique of examination:

- Sagittal T1 & T2 WIs.
- Axial GR/T2 WIs

MRI Findings:

- Straightened cervical curvature.
- Normal diameters of the bony cervical spinal canal.
- Mild C4-5 & C5-6 posterior disc bulges are seen effacing the anterior subarachnoid space yet not touching the spinal cord with no significant neural compromise .
- Normal MRI appearance of the cervical spinal cord and the cranio-cervical junction with no evidence of cord malacia, edema or syrinx formation.
- Normal marrow signal of the imaged vertebrae.
- No paraspinal signal abnormalities.

Conclusion:

- **Mild C4-5 & C5-6 posterior disc bulges, with no MRI evidence of neural compromise.**

II-MRI OF THE DORSAL SPINE

MRI Findings:

- Preserved dorsal kyphosis.
- Normal dimensions of the dorsal spinal canal.
- No MRI evidence of disc bulge or herniation.
- Normal size and signal intensity of the dorsal spinal cord.
- Normal marrow signal intensity pattern of the dorsal vertebrae.
- No abnormal paraspinal soft tissue lesions.

OPINION:

- **No MRI evidence of significant disc lesions or neural compromise.**

Dr, Hala Sayed , MSc

Dr. Sherif Sameh, M.D.



Patient Name: Heba Mohamed Ahmed Hassan	Date : 26/10/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 36 Year(s)
Medical ID: 2685/2023	Pat. ID: 39784

سونا بطن

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average size (about 17.5 cm), displaying homogenous bright echopattern with smooth surface.
- No intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention with normal wall thickness, no stones or diverticulae. CBD is patent and not dilated.

Spleen: -

- Average in size, displaying homogenous echopattern with no focal lesions.

Kidneys: -

- Are of normal size , well differentiated & with no gross stones, or space occupying lesion could be seen.
- **Para-aortic and pancreatic area:** normal echopattern with no focal lesions.
- **No Ascites.**

Urinary bladder: empty.

Uterus & adnexa: no gross abnormalities could be detected.

Colonic gaseous distension.

Opinion:

- Mild fatty hepatomegaly.
- Colonic gaseous distension.

Best regards
Dr. Eman Ebeid. MD

Signature Dr. Eman Mahmoud Ali
PM 1:11 2024/10/26



Patient Name: Heba Mohamed Ahmed Hassan	Date : 26/10/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 36 Year(s)
Medical ID: 2685/2023	Pat. ID: 39784

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- ACR:D
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Youmna tarek

26/10/2024 1:36 PM



Date : 11/11/2023	Patient Name: Heba Mohamed Ahmed Hassan
Age : 35 Year(s)	Exam : Neck arterial and doppler
Pat. ID: 39784	Medical ID: 2665/2023

Color Duplex scanning of the porta cath revealed: -

- *Porta cath is seen at the right infra clavicular region at right subclavian vein with no surrounding hematoma or collection could be seen .*
- *No evidence of significant stenotic or occlusive lesions.*
- *Sub clavian and axillary vessels shows average flow velocity and wave pattern on both sides.*

Best Regards,

Dr/ Hayam Ali, FEBR



Exam Information:

<u>Patient Name:</u> Heba Muhammed	<u>Date of birth:</u> 01/03/1988
<u>Arabic Name:</u> هبة محمد	<u>Patient Gender:</u> F
<u>Patient RIS ID:</u> 344279 <u>Patient HIS ID:</u> 123026322	<u>Exam Date:</u> 18/12/2023
<u>Written by:</u> Dr. Hoda Fathy	<u>Approved by:</u> Dr. Hoda Fathy

Nuclear Medicine Unit

وحدة الطب النووي

^{99m}Tc -MDP BONE SCAN

Brief history: 35 years old female patient diagnosed with managed breast cancer.

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP in the following manner.

Report:

- ☐ No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- ☐ Homogeneous tracer distribution noted over axial and appendicular skeleton.
- ☐ Both kidneys are normally visualized.

Conclusion:

Negative study for the detection of any definite metastatic osseous deposits.



Date : 02/03/2024	Patient Name: Heba Mohamed Ahmed Hassan
Age : 36 Year(s)	Exam : ultrasound BREAST (BOTH SIDE)
	Medical ID: 2685/2023

Name: Mrs Hoda Sayed Mohamed .
Date : 2 March 2024

Soft tissue ultrasound examination of both breast
revealed: -

- Status post left MRM showing :
- Operative bed linear seroma measuring about 5cm at its maximum width, yet no solid mass lesions.
- No pathological regional; LNs.
- **Right breast:**
- Predominantly fibro-glandular breast parenchyma with no areas of architectural distortion.
- Right UOQ few anechoic cysts.
- No suspicious masses could be detected.
- No suspicious clusters of microcalcifications could be detected.
- Intact skin and subcutaneous tissues.
- No evidence of pathological axillary lymphadenopathy .

OPINION:

- Status post left MRM showing no solid mass lesions or no pathological regional LNs.
- Right UOQ few anechoic cysts

Dr/hasama mohamed

21/02/2024

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Date : 02/03/2024	Patient Name: Heba Mohamed Ahmed Hassan
Age : 36 Year(s)	Exam : Neck arterial and doppler
	Medical ID: 2685/2023

COLOR DUPLEX OF PORTACATH REVEALED

- Porta Cath is seen at right infraclavicular region at right subclavian vein with no surrounding hematoma or collection.
- No evidence of significant stenotic or occlusive lesions.
- Subclavian and axillary vessels show average flow velocity

Best Regards,
Dr Dina Gaber, FEBR,

Not Approved



S C A N

Name: Heba Mohamed Ahmed Hassan

ID: 992786

Referred by: Prof.

Date: 06/07/2023

High Field Open (1.5T) Dynamic MR-Mammography of Both Breasts

Technique :

- Axial T2 (plate 1).
- Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the right breast (plate 3).
- Sagittal postcontrast 3D TFE of the left breast (plate 4).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5).
- Time intensity curves (plate 6).

Findings:

- Bilateral glandular condensation and evidences of bilateral homogenous glandular enhancement.
- Scattered tiny sized breast cysts in both breasts .
- Left breast upper outer and lower outer quadrant large infiltrative masses surrounding by enhancing satellite lesions extending between the 12:00 to 5:00 locations , the masses measure 50x40 mm , 6x24 mm and 38x33 mm in diameters , the lesions are low signal in T2 and STAIR with intense mass like enhancement . the kinetic data obtained indicates a malignant pattern , rapid rise followed by wash out and plateau , type 2b& 3 curves .the LOQ lesion is seen surrounded by architecture distortion and associated with skin tapering and retraction.
- Pathologically enlarged right axillary lymph nodes the largest measures 22x17 mm in diameters .
- No right breast masses and no architecture distortion.
- Normal skin thickness and contour of the right breast.
- There is multiple likely non-specific right axillary lymphadenopathy.

OPINION :

Left breast UOQ & LOQ locally advanced multifocal carcinoma, categorized as BIRADS 5 based on the morphological and the kientic data.

Right breast fibroadenosis and tiny cysts , BIRADS 2.

Prof. Naglaa Abdel Razek.MD



19911



alboragscan.com



جميع أنواع الأشعة التشخيصية

Visit Number: 71023514169

Client Name:

Patient Name: هبة محمد احمد

Referred by Prof:

Age / Sex: 35 year / Female

Exam Date:

18 Jul 2023

Patient ID: 2131748481

Printed:

7/18/2023 10:28:11 PM

CT

MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION:

Clinical data: Newly diagnosed breast cancer for metastatic work up.

Findings:

I) CHEST:

- Left breast UOQ irregular enhancing mass lesion measuring about 5x2.5 cm associated with few prominent left axillary (level 1) lymph nodes; the largest measures about 7 mm in maximum short axis diameter (for sono-mammographic correlation).
- Prominent left internal mammary lymph node measuring about 4 mm in short axis diameter.
- Bilateral upper and right lower lobar apical fine reticulo-nodular infiltrations associated with bilateral lower lobar tiny calcified pulmonary nodules (likely sequel of old granulomatous infection).
- Clear rest of both lung fields. No pulmonary masses, areas of consolidation or cavitory lesions.
- No pleural or pericardial effusion.
- No gross cardiomegaly.
- No pathologically enlarged hilar or mediastinal lymph nodes.

II) ABDOMEN & PELVIS:

- The liver is of average size showing segment (IVb) patchy are of non-enhancement (likley pseudo-lesion).
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.



00 شارع عبدالمنعم رياض، برج الأطباء، الدور ٢

09 شارع شبرا

٦٧ شارع الميرغني

محطة التعاون

كابيتال بزنس بارك، مبني B5، الدور ٢

٣٠ شارع الجزائر

٢٢ شارع عبد الرازق السنهوري

فيصل

الشيخ زايد

المعادي

مدينة نصر

CT-Scan

[[DOCTYPE:CT_SCAN]]

Patient Name: هبة محمد احمد
Age / Sex: 35 year / Female
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CT

- Normal size & parenchymal texture of the spleen with no focal lesions seen. A splenule is seen related to the lower pole of the spleen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus with posterior wall well defined hypodense lesion measuring about 1.4 x 0.9 cm (likely fibroid, for US correlation).
- Right adnexal multiple foci of coarse calcifications.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

OPINION:

- No current CT evidence of pulmonary or hepatic metastatic deposits.

Dr.Abdulrahman Ibrahim

Dr. Sally Fouad, MD



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٢٠ شارع الجزائر
٢٢ شارع عبد الرازق السنهوري

فيصل
الشيخ زايد
المعادي
مدينة نصر

Nuclear Medicine Unit

خطابات أورام

Patient Name: Heba Mohamed Ahmed

Date: 11/5/2025

Referring physician: Dr. T. El-Mohamed
99mTc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No active osseous metastatic lesions could be seen all over the surveyed body .
- Spondylo degenerative changes are seen at the spine ,
- Both kidneys are normally seen.

Conclusion:

No active osseous metastatic lesion could be seen all over the surveyed body ,

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

DR/ Mohamed Emam



Echo report

Name:

أحمد محمد

Date: 23/12/2023

MR: 2685

23

EF	69%	55-75%	PWT	0.73	
LVEDD	4.53	3.5-5.8cm	AO.R	1.89	2-3.7cm
LVESD	2.76	2-4cm	LA	2.60	2-4cm
FS	39%	25-45%	RV	N	0.7-2.3cm(supine)
SWT	0.73	0.6-1.1cm	AO.CUSP	N	1.5-2.6cm

LV dimensions not

LV wall thickness not

LV diastolic function not

SWMA no gross SWMA

LA and Aortic root dimensions not

Mitral Valve not, T1M2 MR

Aortic valve not, no PG

Tricuspid valve not, no PG

Pulmonary valve not, no PG

Right side and systolic function not Dilated, TAPSE 23

Intracardiac masses and effusion no effusion or mass

D/ Msc cardiology

دكتور محمد



Echo report

Name:

اسم المريض

Date: 13/1/2024

MR: 2680

23

EF	66%	55-75%	PWT	0.59	
LVEDD	4.74	3.5-5.8cm	AO.R	1.83	2-3.7cm
LVEDS	3.01	2-4cm	LA	3.01	2-4cm
FS	36%	25-45%	RV	N	0.7-2.3cm(supine)
SWT	0.91	0.6-1.1cm	AO.CUSP	N	1.5-2.6cm

LV dimensions

Not

LV wall thickness

Not

LV diastolic function

No

SWMA

No SWMA

LA and Aortic root dimensions

Not

Mitral Valve

Not

1. NO PC

Aortic valve

Not

2. NO PC

Tricuspid valve

Not

1. NO PC

Pulmonary valve

Not

2. NO PC

Right side and systolic function

Not Dilated, TAPSE 2

Intracardiac masses and effusion

No effusion, No masses

D/ Msc cardiology

دراهمي لقرن

Patient Name	: الأستاذ / هبة محمد احمد	Request No.	: 13008838
Patient No.	: 28803010102627	Request Date	: 09/07/2023
Path No.	: 2145/23	Verification Date	: 16/07/2023
Sex	: Female	Financial Category	: اورام دار السلام - ميانره
Age	: 35 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 09/07/2023, 11:00 am.

Gross Examination:

- Five grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
 - PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
 - Her2: No membrane reaction in tumor cells (score 0).
 - Ki67: 20% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 20% of tumor cells.

Verified by :

Dr.Marwa mahmoud

Printing Date : 18/07/2023 09:24:13

Page 1 of 2

Printed By : marwa mohamed youss

Patient Name	: الاستاذة / هبة محمد احمد	Request No.	: 13002900
Patient No.	: 28803010102627	Request Date	: 29/01/2024
Path No.	: 403/24	Verification Date	: 06/02/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادئ
Age	: 35 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer (post NACT) .
Nature of the specimen: Left MRM

Gross Examination:

- Procedure: Left MRM
- measured 29x17x5 cm with covering skin ellipse measured 26x10cm & attached axilla 8x6 cm .
- Tumor extent : A greyish white tumour bed site
- Size: 3.5x3x1 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 0.2 cm away from deep margin ,
- Lymph nodes number and size: 14 L.Ns up to 2.5x1 cm .

Microscopic examination:

- Histological type: No residual viable tumor tissue detected
- Ductal carcinoma in situ: No
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/14)
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic Complete response

Diagnosis:

Left MRM (post NACT) :

No residual viable tumor tissue detected.

- Free margins
- Free nipple and areola
- Free L.Ns (0/14)
- Pathologic Complete Response
- Stage : ypT0 , ypN0

Verified by :

Dr.Heba Hasan



Patient Name : الأستاذة / حبه محمد احمد

Patient No. : 28803010102627

Path No. : 403/24

Sex : Female

Age : 35 Y

Referring Doc :

Request No. : 13002900

Request Date : 29/01/2024

Verification Date : 06/02/2024

Financial Category : اورام دار السلام - مبانء

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Prof. Dr. Gamal Abdelatty

Dr. Israa Magdy

Verified by :

Dr.Heba Hasan

Printing Date : 10/02/2024 13:06:35

Page 2 of 2

Printing By : marwa mohamed youss



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